



Patient Information	Specimen Information	Client Information
GOPALAKRISHNAN, SRIKRISHNA DOB: 08/11/1978 AGE: 47 Gender: M Fasting: Y Phone: 408.416.6993 Patient ID: 0B367884 Health ID: 8573021193699820	Specimen: SZ015940Y Requisition: 0201920 Lab Ref #: 90876EA4DE1D427D Collected: 03/18/2026 / 10:12 PDT Received: 03/19/2026 / 01:20 PDT Reported: 03/23/2026 / 22:20 PDT	Client #: 73929412 MAIL992 DAMASCO, LEO JUNCTION 440 N BARRANCA AVE # 3811 COVINA, CA 91723-1722

COMMENTS: PATIENT MUST BE FASTING
FASTING: YES

Test Name	In Range	Out Of Range	Reference Range	Lab
IRON, TIBC AND FERRITIN PANEL				
IRON AND TOTAL IRON				UL
BINDING CAPACITY				
IRON, TOTAL	125		50-180 mcg/dL	
IRON BINDING CAPACITY	328		250-425 mcg/dL (calc)	
% SATURATION	38		20-48 % (calc)	
FERRITIN	107		38-380 ng/mL	UL
THYROID PANEL WITH TSH				
THYROID PANEL				UL
T3 UPTAKE	34		22-35 %	
T4 (THYROXINE), TOTAL	6.0		4.9-10.5 mcg/dL	
FREE T4 INDEX (T7)	2.0		1.4-3.8	
TSH	2.72		0.40-4.50 mIU/L	UL
TESTOSTERONE, FREE, BIOAVAILABLE AND TOTAL, MS				
ALBUMIN	4.6		3.6-5.1 g/dL	EZ
SEX HORMONE BINDING				EZ
GLOBULIN	21		10-50 nmol/L	
TESTOSTERONE, TOTAL, MS	328		250-1100 ng/dL	EZ
Men age 18 or above, with clinically significant hypogonadal symptoms and total testosterone values repeatedly in the range of 200-300 ng/dL or less, may benefit from testosterone treatment after adequate risk and benefits counseling.				
For additional information, please refer to http://education.questdiagnostics.com/faq/TotalTestosteroneL CMSMS (This link is being provided for informational/educational purposes only.)				
This test was developed and its analytical performance characteristics have been determined by Quest Diagnostics. It has not been cleared or approved by the FDA. This assay has been validated pursuant to the CLIA regulations and is used for clinical purposes.				
TESTOSTERONE, FREE AND				EZ
BIOAVAILABLE				
TESTOSTERONE, FREE	58.7		46.0-224.0 pg/mL	
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TESTOSTERONE, BIOAVAILABLE	123.2		110.0-575.0 ng/dL	
LIPOPROTEIN (a)		87 H	nmol/L	EN
		Reference Range	<75	
		Risk:		
		Optimal	<75	
		Moderate	75-125	



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Test Name	In Range	Out Of Range High	Reference Range >125	Lab
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Cardiovascular event risk category cut points (optimal, moderate, high) are based on Tsimika S. JACC 2017;69:692-711.

LIPID PANEL WITH RATIOS

CHOLESTEROL, TOTAL

HDL CHOLESTEROL

TRIGLYCERIDES

LDL-CHOLESTEROL

Reference range: <100

51

74

201 H
133 H

<200 mg/dL

> OR = 40 mg/dL

<150 mg/dL

mg/dL (calc)

UL

UL

UL

UL

Desirable range <100 mg/dL for primary prevention;
<70 mg/dL for patients with CHD or diabetic patients
with > or = 2 CHD risk factors.

LDL-C is now calculated using the Martin-Hopkins calculation, which is a validated novel method providing better accuracy than the Friedewald equation in the estimation of LDL-C.

Martin SS et al. JAMA. 2013;310(19): 2061-2068
(<http://education.QuestDiagnostics.com/faq/FAQ164>)

CHOL/HDLC RATIO

LDL/HDL RATIO

3.9

2.6

<5.0 (calc)

(calc)

Below Average Risk:

Average Risk:

Moderate Risk:

High Risk:

<2.28

2.29-4.90

4.91-7.12

>7.13

UL

UL

NON HDL CHOLESTEROL
150 H

<130 mg/dL (calc)

UL

For patients with diabetes plus 1 major ASCVD risk factor, treating to a non-HDL-C goal of <100 mg/dL (LDL-C of <70 mg/dL) is considered a therapeutic option.

HS CRP

0.9

mg/L

UL

Reference Range

Optimal <1.0

Jellinger PS et al. Endocr Pract.2017;23(Suppl 2):1-87.

For ages >17 Years:

hs-CRP mg/L Risk According to AHA/CDC Guidelines

<1.0 Lower relative cardiovascular risk.

1.0-3.0 Average relative cardiovascular risk.

3.1-10.0 Higher relative cardiovascular risk.

Consider retesting in 1 to 2 weeks to exclude a benign transient elevation in the baseline CRP value secondary to infection or inflammation.

>10.0 Persistent elevation, upon retesting, may be associated with infection and inflammation.

Pearson TA, Mensah GA, Alexander RW, et al. Markers of inflammation and cardiovascular disease: application to clinical and public health practice: A statement for healthcare professionals from the



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Test Name	In Range	Out Of Range	Reference Range	Lab
Centers for Disease Control and Prevention and the American Heart Association. Circulation 2003; 107(3): 499-511.				
HOMOCYSTEINE		13.8 H	< or = 13.5 umol/L	UL
Homocysteine is increased by functional deficiency of folate or vitamin B12. Testing for methylmalonic acid differentiates between these deficiencies. Other causes of increased homocysteine include renal failure, folate antagonists such as methotrexate and phenytoin, and exposure to nitrous oxide. Selhub J, et al., Ann Intern Med. 1999;131(5):331-9.				
APOLIPOPROTEIN B		106 H	mg/dL Reference Range <90	EN
Risk Category: Optimal <90 Moderate 90-129 High > or = 130				
A desirable treatment target may be <80 mg/dL or lower depending on the risk category of the patient including patients on lipid lowering therapies, patients with ASCVD, diabetes with >1 risk factors, Stage 3 or greater CKD with albuminuria, or heterozygous familial hypercholesterolemia. ApoB relative risk category cut points are based on AACE/ACE and ACC/AHA recommendations (Grundy SM, et al. 2019. doi:10.1016/j.jacc.2018.11.002; Handelsman Y, et al. 2020. doi:10.4158/CS-2020-0490).				
COMPREHENSIVE METABOLIC PANEL				UL
GLUCOSE	95		65-99 mg/dL	
Fasting reference interval				
UREA NITROGEN (BUN)	16		7-25 mg/dL	
CREATININE	1.04		0.60-1.29 mg/dL	
EGFR	89		> OR = 60 mL/min/1.73m2	
BUN/CREATININE RATIO	15		6-22 (calc)	
SODIUM	141		135-146 mmol/L	
POTASSIUM	4.5		3.5-5.3 mmol/L	
CHLORIDE	103		98-110 mmol/L	
CARBON DIOXIDE	30		20-32 mmol/L	
CALCIUM	9.9		8.6-10.3 mg/dL	
PROTEIN, TOTAL	7.4		6.1-8.1 g/dL	
ALBUMIN	4.7		3.6-5.1 g/dL	
GLOBULIN	2.7		1.9-3.7 g/dL (calc)	
ALBUMIN/GLOBULIN RATIO	1.7		1.0-2.5 (calc)	
BILIRUBIN, TOTAL	0.7		0.2-1.2 mg/dL	
ALKALINE PHOSPHATASE	45		36-130 U/L	
AST	36		10-40 U/L	
ALT	30		9-46 U/L	



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Test Name	In Range	Out Of Range	Reference Range	Lab
HEMOGLOBIN A1c WITH eAG				UL
HEMOGLOBIN A1c		5.8 H	<5.7 %	

For someone without known diabetes, a hemoglobin A1c value between 5.7% and 6.4% is consistent with prediabetes and should be confirmed with a follow-up test.

For someone with known diabetes, a value <7% indicates that their diabetes is well controlled. A1c targets should be individualized based on duration of diabetes, age, comorbid conditions, and other considerations.

This assay result is consistent with an increased risk of diabetes.

Currently, no consensus exists regarding use of hemoglobin A1c for diagnosis of diabetes for children.

eAG (mg/dL)	120	mg/dL	
eAG (mmol/L)	6.6	mmol/L	
URIC ACID	6.3	4.0-8.0 mg/dL	UL
Therapeutic target for gout patients: <6.0 mg/dL			
BILIRUBIN, FRACTIONATED			UL
BILIRUBIN, TOTAL	0.7	0.2-1.2 mg/dL	
BILIRUBIN, DIRECT	0.1	< OR = 0.2 mg/dL	
BILIRUBIN, INDIRECT	0.6	0.2-1.2 mg/dL (calc)	
GGT	27	3-95 U/L	UL
T3, FREE	3.5	2.3-4.2 pg/mL	UL
THYROID PEROXIDASE AND THYROGLOBULIN ANTIBODIES			
THYROGLOBULIN ANTIBODIES	<1	< or = 1 IU/mL	EN
THYROID PEROXIDASE ANTIBODIES	1	<9 IU/mL	EN
IGF 1, LC/MS	88	52-328 ng/mL	EZ
Z SCORE (MALE)	-1.0	-2.0 - +2.0 SD	

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SED RATE BY MODIFIED WESTERGREN	2	< OR = 15 mm/h	UL
CBC (INCLUDES DIFF/PLT)			UL
WHITE BLOOD CELL COUNT	4.4	3.8-10.8 Thousand/uL	
RED BLOOD CELL COUNT	5.16	4.20-5.80 Million/uL	
HEMOGLOBIN	15.6	13.2-17.1 g/dL	
HEMATOCRIT	46.5	39.4-51.1 %	
MCV	90.1	81.4-101.7 fL	
MCH	30.2	27.0-33.0 pg	
MCHC	33.5	31.6-35.4 g/dL	
RDW	12.0	11.0-15.0 %	
PLATELET COUNT	239	140-400 Thousand/uL	
MPV	10.2	7.5-12.5 fL	



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Test Name	In Range	Out Of Range	Reference Range	Lab
ABSOLUTE NEUTROPHILS	2160		1500-7800 cells/uL	
ABSOLUTE LYMPHOCYTES	1712		850-3900 cells/uL	
ABSOLUTE MONOCYTES	339		200-950 cells/uL	
ABSOLUTE EOSINOPHILS	158		15-500 cells/uL	
ABSOLUTE BASOPHILS	31		0-200 cells/uL	
NEUTROPHILS	49.1		%	
LYMPHOCYTES	38.9		%	
MONOCYTES	7.7		%	
EOSINOPHILS	3.6		%	
BASOPHILS	0.7		%	
URINALYSIS, COMPLETE				UL
COLOR	YELLOW		YELLOW	
APPEARANCE	CLEAR		CLEAR	
SPECIFIC GRAVITY	1.029		1.001-1.035	
PH	6.0		5.0-8.0	
GLUCOSE	NEGATIVE		NEGATIVE	
BILIRUBIN	NEGATIVE		NEGATIVE	
KETONES		TRACE	NEGATIVE	
OCCULT BLOOD	NEGATIVE		NEGATIVE	
PROTEIN	NEGATIVE		NEGATIVE	
NITRITE	NEGATIVE		NEGATIVE	
LEUKOCYTE ESTERASE	NEGATIVE		NEGATIVE	
WBC	NONE SEEN		< OR = 5 /HPF	
RBC	NONE SEEN		< OR = 2 /HPF	
SQUAMOUS EPITHELIAL CELLS	NONE SEEN		< OR = 5 /HPF	
BACTERIA	NONE SEEN		NONE SEEN /HPF	
HYALINE CAST	NONE SEEN		NONE SEEN /LPF	

This urine was analyzed for the presence of WBC, RBC, bacteria, casts, and other formed elements. Only those elements seen were reported.

VITAMIN B12/FOLATE, SERUM PANEL				UL
VITAMIN B12	398		200-1100 pg/mL	

Please Note: Although the reference range for vitamin B12 is 200-1100 pg/mL, it has been reported that between 5 and 10% of patients with values between 200 and 400 pg/mL may experience neuropsychiatric and hematologic abnormalities due to occult B12 deficiency; less than 1% of patients with values above 400 pg/mL will have symptoms.

FOLATE, SERUM	16.6		ng/mL Reference Range Low: <3.4 Borderline: 3.4-5.4 Normal: >5.4	
DHEA SULFATE	266		61-442 mcg/dL	EN
INSULIN	9.2		uIU/mL	EN
		Reference Range	< or = 18.4	
		Risk:		
		Optimal	< or = 18.4	
		Moderate	NA	



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Test Name	In Range	Out Of Range	Reference Range	Lab
	High		>18.4	

Adult cardiovascular event risk category cut points (optimal, moderate, high) are based on Insulin Reference Interval studies performed at Quest Diagnostics in 2022.

CORTISOL, TOTAL	5.8	mcg/dL	UL
Reference Range: For 8 a.m.(7-9 a.m.) Specimen: 4.0-22.0			
Reference Range: For 4 p.m.(3-5 p.m.) Specimen: 3.0-17.0			
* Please interpret above results accordingly *			

FSH	3.3	1.4-12.8 mIU/mL	UL
LH	1.9	1.5-9.3 mIU/mL	UL
PROGESTERONE	0.5	<1.4 ng/mL	UL
PROLACTIN	3.5	2.0-18.0 ng/mL	UL
ESTRADIOL	<30	< OR = 39 pg/mL	UL

Reference range established on post-pubertal patient population. No pre-pubertal reference range established using this assay. For any patients for whom low Estradiol levels are anticipated (e.g. males, pre-pubertal children and hypogonadal/post-menopausal females), the Quest Diagnostics Nichols Institute Estradiol, Ultrasensitive, LCMSMS assay is recommended (order code 30289).

Please note: patients being treated with the drug fulvestrant (Faslodex(R)) have demonstrated significant interference in immunoassay methods for estradiol measurement. The cross reactivity could lead to falsely elevated estradiol test results leading to an inappropriate clinical assessment of estrogen status. Quest Diagnostics order code 30289-Estradiol, Ultrasensitive LC/MS/MS demonstrates negligible cross reactivity with fulvestrant.

PSA (FREE AND TOTAL)			EN
PSA, TOTAL	0.4	< OR = 4.0 ng/mL	
PSA, FREE	0.1	ng/mL	
PSA, % FREE		25 L	
		>25 % (calc)	

PSA(ng/mL)	Free PSA(%)	Estimated(x) Probability of Cancer(as%)
0-2.5	(*)	Approx. 1
2.6-4.0(1)	0-27(2)	24(3)
4.1-10(4)	0-10	56
	11-15	28
	16-20	20
	21-25	16
	>or =26	8
>10(+)	N/A	>50

References:(1)Catalona et al.:Urology 60: 469-474 (2002)
(2)Catalona et al.:J.Urol 168: 922-925 (2002)

Free PSA(%)	Sensitivity(%)	Specificity(%)
< or = 25	85	19



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Test Name	In Range	Out Of Range	Reference Range	Lab
	< or = 30	93	9	
	(3)Catalona et al.:JAMA 277: 1452-1455 (1997)			
	(4)Catalona et al.:JAMA 279: 1542-1547 (1998)			

- (x)These estimates vary with age, ethnicity, family history and DRE results.
(*)The diagnostic usefulness of % Free PSA has not been established in patients with total PSA below 2.6 ng/mL
(+)In men with PSA above 10 ng/mL, prostate cancer risk is determined by total PSA alone.

The Total PSA value from this assay system is standardized against the equimolar PSA standard. The test result will be approximately 20% higher when compared to the WHO-standardized Total PSA (Siemens assay). Comparison of serial PSA results should be interpreted with this fact in mind.

PSA was performed using the Beckman Coulter Immunoassay method. Values obtained from different assay methods cannot be used interchangeably. PSA levels, regardless of value, should not be interpreted as absolute evidence of the presence or absence of disease.



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Endocrinology

Test Name	Result	Reference Range	Lab
VITAMIN D,25-OH,TOTAL,IA	72	30-100 ng/mL	UL
Vitamin D Status 25-OH Vitamin D: Deficiency: <20 ng/mL Insufficiency: 20 - 29 ng/mL Optimal: > or = 30 ng/mL For 25-OH Vitamin D testing on patients on D2-supplementation and patients for whom quantitation of D2 and D3 fractions is required, the QuestAssureD(TM) 25-OH VIT D, (D2,D3), LC/MS/MS is recommended: order code 92888 (patients >2yrs). For additional information, please refer to http://education.QuestDiagnostics.com/faq/FAQ199 (This link is being provided for informational/ educational purposes only.) Physician Comments:			

PERFORMING SITE:

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